

An invitation to collaborate

Two mandates for Dr Chisom Adaobi Nri-Ezedi

A virtual paediatric haemato-oncology service to design and lead, and a clinical protocol mandate on a live paediatric and neonatal engagement. Two ways to bring your clinical depth to bear at scale, under the Consult for Africa banner.

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For: Dr Chisom Adaobi Nri-Ezedi

From: Dr Debo Odulana, Founding Partner, Consult for Africa



1. Why I am writing

Chisom, you sit in a rare position. You are a consultant paediatrician with genuine subspecialty depth in haemato-oncology and endocrinology, a Reader and acting head of department, and at the same time a self-taught data scientist who ships real clinical software. Very few clinicians anywhere combine that clinical authority with the instinct to build. That combination is exactly what Consult for Africa exists to put to work.

This note sets out two mandates I would like you to take on with us. The first is a service to design and lead as your own. The second is delivery work on a live engagement where your expertise is needed now. You can take one or both.

2. How Consult for Africa works

Consult for Africa is a healthcare transformation partner working with hospitals, clinics, and health businesses across Nigeria. We deliver through a faculty of senior clinicians and operators who lead defined mandates under the firm's banner, with the firm handling the commercials, the client relationship, and the platform behind you. You bring the clinical authority; we bring the structure, the reach, and the delivery engine.

In short: you do the work you are uniquely good at, at a scale you could not reach alone, and we make it a business rather than a favour.

3. Mandate one: a virtual paediatric haemato-oncology service

Yours to design, build, and lead.

Childhood cancer and blood disorders in Nigeria are concentrated in a handful of centres, while the children and the referring doctors are everywhere. A virtual service closes that distance. I would like you to design and implement it as a Consult for Africa service line, with you as its clinical lead.

What it could include:

- Remote consultation and second-opinion pathways for referring paediatricians and family doctors who do not have a haem-oncologist to hand.
- A virtual tumour board and shared-care model, so a child can be co-managed close to home under specialist oversight.
- Protocolised supportive care and treatment guidance for centres delivering care locally, with clear escalation and referral criteria.
- The digital layer to make it work, an area where your own building instinct is a real advantage rather than an afterthought.
- A workable access and payment model, including how diaspora and senior specialists could contribute sessions through our workforce platform.

We would start with a short design phase: the clinical model, the operating model, and a costed pilot. From there we build and run it together. This is meant to be yours to own and grow, not a task to complete.



4. Mandate two: clinical protocol design on a live engagement

Immediate, well-defined delivery work.

We are advising a young, ambitious paediatric and neonatal facility in Lagos that is deliberately building its clinical systems early, before growth outpaces them. Part of that work is authoring the clinical protocols and nursing standards of work that make reliable care automatic rather than dependent on individual diligence. It needs a paediatrician of unimpeachable standing to lead it, and I would like that to be you.

The scope would cover:

- Core paediatric and neonatal clinical protocols and care pathways, written to a best-practice standard the team can grow into.
- Nursing standards of work and shift-level routines, including emergency-readiness standards such as a checklist-governed crash cart.
- Clear escalation, referral, and handover pathways.
- A light-touch review cadence so the protocols stay living documents, not shelfware.

You would work alongside our engagement lead on the project, focused purely on the clinical-standards layer. The client is a strong young facility getting the foundations right first, which is precisely the kind of forward-looking work that suits your standing. Full details follow once we agree to proceed and put the usual confidentiality in place.



5. Why you, specifically

What the work needs	What you bring
Subspecialty authority in paediatric haem-oncology	FWACP, MD Paediatrics, PhD in clinical oncology, and a paediatric haematology-oncology and endocrinology practice
Credibility to set clinical standards others follow	Reader and acting head of department, subspecialty training coordinator, honorary consultant at a teaching hospital
Ability to build the digital service, not just specify it	Certified data analyst and self-taught developer who has shipped clinical apps and won global data competitions
A mission that matches the work	A stated commitment to widening access to specialist care for African children

6. How we would structure it

Both mandates run as Consult for Africa engagements, with clear scope and clear terms agreed up front. The protocol work is a defined deliverable and would be scoped and priced as such. The virtual service is a build, so we would structure it to reflect that you are creating and leading a lasting service line, not billing hours. We will agree the commercial shape together, and I want it to feel worth your while and aligned to the value you create.

Practical points, such as time commitment around your university duties, will be built around your availability. Nothing here asks you to choose between this and your post at the university. It is designed to sit alongside it.

7. Next steps

If either mandate appeals, and I hope both do, let us find a short call. For the virtual service we would agree the design phase and its scope. For the protocol work we would put confidentiality in place, share the detail, and set you up with the engagement lead. I would be glad to move quickly.

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